

On stress/social stress

Types of Stressors

In recent years, sociologists have distinguished between two major types of stressors: specific **life events** and more enduring life problems called **chronic strains**.

Life Events. Life events are important specific events or experiences that interrupt an individual's usual activities and require some adjustment. A distinction is made between anticipated (or scheduled) life events (such as marriage, divorce, and the beginning or ending of a school year) and unanticipated (unscheduled) life events (such as the death of a loved one, a sudden failure, the sudden loss of a job, or learning of a terminal illness). Recent research has begun to explore the importance of anticipatory events—those that might happen in the future (for example, failing out of school or being a victim of a crime). The very anticipation of such events might in itself be a stressor.

In order to determine the effects of these specific life events on stress level, researchers have employed three kinds of techniques: (1) studies of the psychiatric effects of specific events such as reactions to combat and natural and human disasters, (2) comparison of the number and types of life events experienced by psychiatric patients prior to their hospital admission to those for a nonpatient control group, and (3) general population surveys examining the relationship between life events, stress, and illness. Researchers have developed a variety of specific scales to measure exposure to stressful life events. One popular scale—the Social Readjustment Rating Scale (Holmes and Rahe, 1967)—contains a list of 43 events that were evaluated by a panel of judges as to the level of readjustment that each required. The most stressful life events were identified as being the death of a spouse, a divorce, marital separation, and a jail term. At the other end of the scale were minor violations of the law, Christmas, vacation, and change in eating habits.

Does experiencing undesirable life events negatively impact health? Yes, and sometimes the event is traumatizing, but often the effect is not large, and, in most cases, it does not persist

over a long period of time. Researchers do continue to detect a relationship between adverse life events and certain depressive disorders (Dohrenwend, 2000), but the effects are not large and generally dissipate within three months (Avison and Turner, 1988).

Chronic Strains. The second major type of stressor—now often referred to as chronic strains—refers to the relatively enduring problems, conflicts, and threats that people typically face in their daily lives. The most common bases for these types of stressors are family problems with spouse, parents, or children; love or sex problems; problems on the job or in school; and problems in any site that involve competition. A meaningful way to organize these chronic stressors is to focus on problems that occur within the boundaries of major social roles and role sets. These are likely to be important problems because the relationships that exist in role sets are usually enduring. Because they also tend to be extremely important relationships (e.g., spouse, child, boss, and teacher), strains that develop are likely to be of great significance to the individual (Pearlin, 1989).

Pearlin (1989:245) uses the concept of “role strain” to refer to “the hardships, challenges, and conflicts or other problems that people come to experience as they engage over time in normal social roles.” The five most common types of role strain are discussed next.

1. **Role overload** occurs when the combination of all the role demands placed on an individual exceed that individual's ability to meet them. Within the workplace, there is evidence that work overload is most likely to be felt by those at opposite ends of the spectrum: salaried, white-collar workers and the least-skilled, blue-collar workers. For different reasons, both may feel little control over work demands—an important predictor of job stress. Excessive workload may also be experienced by the homemaker in overseeing house maintenance, food preparation, and child-rearing functions, as well as increasingly playing the caregiver role for parents



One of the leading causes of stress is time pressure: busy schedules with constant deadlines. In many cultures around the world, there is considerably less pressure than in the United States to overload schedules and to have constant meeting deadlines.

unable to live independently. Primary caregivers and those caring for elders with significant needs must often resort to taking unpaid leave, reducing work hours, rearranging their work schedule, or even leaving the workforce altogether.

Perception of role overload is also influenced by the level of economic return on one's labors. Brenner (1973) has shown that both the absolute level of economic reward and the perceived fairness with changes in the amount of reward influence level of stress. Teaching and nursing are good illustrations of careers that pay more than the national average but are stressful, in part, because the level of training required and the amount and intensity of work demanded are not always commensurately rewarded. Likewise, a frustration of many homemakers is that the value of their contributions is often not rewarded at all—either in monetary fashion or in terms of genuine appreciation.

2. **Interpersonal conflicts within role sets** include problems and difficulties that arise within complementary role sets, such as wife–husband, parent–child, and worker–supervisor, and are the types of strain that often touch people most deeply. Marriage (or engaging in a long-term relationship) is

typically the center of our most intimate relationships, the context of many of our most far-reaching decisions (e.g., children, major purchases, degree of equalitarianism), and the role set in which many spend the most time. Therefore, it has the potential for great bliss as well as significant interpersonal conflict. Rates of separation and divorce, emotional and physical abuse within families, and reported levels of marital dissatisfaction all reflect high levels of stress. Pearlin pinpointed one aspect of this conflict:

One of the more common elements of discord—and one of the more stressful—involves a breakdown in reciprocity. By reciprocity and its failure, I mean the sense of inequity people have about their marriages . . . people see themselves in marital relationships where . . . they invest more than their partner in the relationship, and are more considerate of their partners than they think their partners are of them. (1983:10)

Pearlin (1983) identifies other common sources of strain in marriages: (1) a perception that the spouse does not recognize or accept “quintessential” elements of one’s self—that he or she fails to authenticate what is judged to be an especially prized aspect of the self-image; (2) a belief that the spouse is failing to fulfill basic marital expectations such as

Sociological research increasingly shows that participation in positive social relationships with others is an important contributor to good physical and mental health and can play a significant role in recovering from illness.



Don't draw these pictures on the answer scripts

wage earning or housekeeping; and (3) a feeling that the spouse is failing to provide even minimal levels of affection or that sexual relations are insufficiently satisfying. The lack of physical as well as emotional intimacy clearly relates to marital stress.

3. Interrole conflict occurs when the demands of two or more roles held by a person are incompatible, and the demands cannot simultaneously be met. On a small scale, genuine conflict occurs whenever any health care worker is “on call” and gets called in to the hospital just as he or she is about to participate in a family function (let’s say a one-showing only of a play or dance for which the youngest child has earnestly practiced for months). Being a responsible health care worker and being a loving parent are both very important roles, but on the night in question, the child will be disappointed. In a marriage of two persons equally dedicated to careers, an elderly parent or young child who requires significant attention during the day will force some resolution of an interrole conflict.

4. Role captivity is the term used by Pearlin to describe situations in which an individual is in an unwanted role—he or she feels an obligation to do one thing but prefers to do something else (Pearlin, 1983). A retired person who wishes to continue working and a person working who wishes to retire are both held in role captivity. A college student forced to attend college by parents and a college-age person who wants to go to college but cannot afford it are role captives. Anyone hating his or her job and longing for another is a role captive.

The captive situation can also occur within families. Feeling trapped in an unhappy marriage can be an extremely stressful situation. Sometimes children in families can be role captives as is illustrated by research by Fischer and her colleagues (2000) on the stressfulness of growing up in a family with parental alcoholism.

5. Role restructuring occurs in situations in which long-established patterns or expectations undergo considerable restructuring.

Pearlin (1989) offers such examples as a rebellious adolescent who desires more independence; an apprentice who grows frustrated with his or her mentor as the craft is learned; and adult children who must take on increased responsibilities for aging parents. He notes that the transition can be more difficult when it is forced by circumstances (rather than voluntary effort) and when the transition involves some redistribution of status, privilege, or influence over others. Stress in the workplace seems to be increasing, and part of the explanation for this is anxiety over the possible loss of job (as businesses downsize) and rearranged job responsibilities (to make up for the reduced staff size).

Of course, not all chronic stressors can be related to problems in carrying out one's roles. Pearlin (1989) also refers to "ambient stressors" to identify those that do not attach to any particular role. An example would be living at someplace that is too noisy.

Three final points about chronic strains deserve attention:

1. Chronic strains are a more powerful determinant of depressive disorders and other health problems than are discrete life events. Their persistence, emergence in important areas such as marriage and work, and presence throughout the course of each day give them powerful force within our lives.
2. Valid and reliable measurement of chronic strains (like life events) is complicated. For example, it may be difficult to determine the actual "chronicity" of a strain. Interpersonal conflict within a marriage can rarely be represented as a linear phenomenon—it often ebbs and flows, sometimes swinging back and forth between bliss and misery, and does so with very uneven degrees of intensity. How then does one accurately measure the length of time for which discord has occurred?

Moreover, the specific array of stressors being faced undergoes change during the life course—what is very stressful to an early teen would typically be far different than what is

confronting someone in the later years. Plus, stressors may accumulate over time so that determination of the consequences of any single stressor becomes more difficult. What may seem rather straightforward to measure is actually quite complex (Kessler, Price, and Wortman, 1985).

3. Life events and chronic strains may accumulate over time and often overlap. The occurrence of specific life events may alter the existence or meaning of chronic strains. An example is the effect of sudden job loss (a discrete life event) on division of labor within the household (possibly a chronic strain). Moreover, life events may create new strains or magnify existing strains, as might occur if the sudden job loss created marital discord (Kessler, Price, and Wortman, 1985; Pearlin, 1989).

APPRAISAL OF STRESSORS

Appraisal and the Sociological Perspective

Within sociology, **symbolic interactionism** is a micro-level perspective that focuses on small-scale, everyday patterns of social interaction. Symbolic interactionists believe that social life is comprised of a myriad number of episodes of daily social interactions in which people communicate verbally and nonverbally and engage in a constant process of interpreting others' messages and responding to these interpretations. According to symbolic interactionism, the world is not so much imposed upon the individual, dictating or strongly influencing behavior, as it is created by the individual through the exchange of these verbal and nonverbal symbols. Berger and Luckmann (1967) assigned the term **social construction of reality** to identify this pattern.

A classic example of the symbolic interactionist perspective is found in the work of W.I. Thomas (1863–1947). Thomas recognized that individuals are affected by events only to the extent to which they are perceived. In other words, neither life events nor chronic strains are in and of themselves stressful. They are simply situations or occurrences in which the likelihood of a stressful response is increased.

CHAPTER 11

Complementary and Alternative Medicine

Learning Objectives

- Define “complementary and alternative medicine,” and identify and describe the key characteristics that CAM have in common.
- Discuss the extent to which CAM has become institutionalized in the United States.
- Compare and contrast the origins and historical development of the four CAM approaches examined in this chapter.
- Compare and contrast the relationship with organized medicine of the four CAM approaches examined in this chapter.

Through much of the twentieth century, the scientific medicine paradigm (as described in Chapter 2) was so dominant in the United States that it was referred to as *orthodox* or *conventional* medicine. While alternatives to medical doctors—everything from home remedies to prayer to chiropractors—were frequently used, they were considered to be unorthodox or unconventional medicine. Scientific medicine has been taught almost exclusively in health courses in schools, has been the subject of public health campaigns, and has been the dominant perspective in the medical school curriculum.

THE MEANING OF COMPLEMENTARY AND ALTERNATIVE MEDICINE

Because scientific medicine has been given this societal endorsement, it is rather remarkable that **complementary and alternative medicine (CAM)**—“an array of health care approaches with a history of use or origins outside of mainstream medicine” (National Center for Complementary and Alternative Medicine,

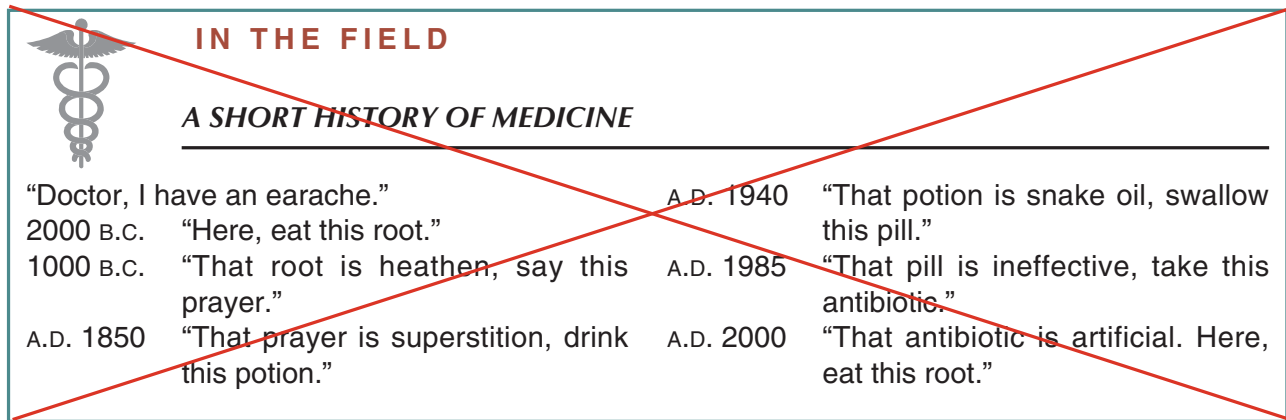
2013:1) has flourished and is today more popular than ever before. The National Center for Complementary and Alternative Medicine (2013) categorizes CAM into:

1. Natural products including herbs, vitamins and minerals, and probiotics [foods or supplements that contain living microorganisms (such as yogurt) that can change the bacterial balance in the human body].
2. Mind and body practices including acupuncture, massage therapy, meditation, movement therapies, relaxation techniques, spinal manipulation, and yoga.

Some important practices such as traditional Chinese healing, folk healing, homeopathy, and naturopathy are in addition to those identified in these two categories.

Goldstein (1999) has extracted five core elements from this wide variety of CAM healing practices:

1. **Holism.** This practice involves treating the patient holistically, that is, considering the entire physical, mental, spiritual, and social



makeup of the patient in diagnosing illness and providing therapeutic care.

2. **The interpenetration of mind, body, and spirit.** While most physicians today recognize the importance of the mind–body connection, CAM places great emphasis on their relationship and generally never treats one without the other.
3. **The possibility of high-level wellness.** Health is viewed as being a very positive physical–emotional state and not just as the absence of symptoms or clinical disease.
4. **Vitalism: life suffused by the flow of energy.** Life is viewed as a type of ecosystem in which the various elements of mind, body, and spirit are united by a force or flow of energy throughout the body.

An amusing overview of the evolution of CAM is provided in the accompanying box, "A Short History of Medicine."

5. **The healing process.** In most forms of CAM, unlike much of conventional medicine, healing is viewed as a cooperative, active process that involves both healer and patient. The healer is a caring and nurturant individual who works "with" instead of "on" patients.

Given the disdain that organized medicine has historically had for CAM (and in many cases the disdain that CAM has had for scientific medicine), the popularity of the alternatives makes an important statement about many people's understanding of health and healing. In fact, Goldner (1999) argues that one of the reasons that many patients choose a CAM technique is

precisely because they feel alienated from the impersonality of conventional medicine, and they prefer a more holistic approach.

SCIENTIFIC MEDICINE AND ALTERNATIVE HEALING

Orthodox Medicine's View of Alternative Healers

Historically, physicians justified their opposition to alternative healing practices in two ways. First, many medical doctors have considered any form of "nonscientific" healing to be quackery—a medically worthless practice—or a danger to public health (if a harmful substance is administered or if people delay seeking conventional care). Their criticism of CAM was viewed as being part of a duty to protect the public's health. Physician-critics acknowledge that some alternative healers make a professional appearance and appear to base their practice on well-articulated (though nonscientific) principles. But, by virtue of offering a healing practice that has not undergone rigorous scientific testing, they are viewed as deluding the public and risking people's health (Angell and Kassirer, 1998).

Second, physicians have expressed concern that some people are fooled into believing the claims of alternative healers. Whether it is due to effective advertising or to appeals made to people who have not been helped by orthodox medicine, users of CAM have sometimes been seen as being unable to distinguish

between legitimate and illegitimate medical care (Beyerstein, 2001).

An alternative view suggests that organized medicine's opposition to alternative healers has been based on perceived self-interest. By persuading the public (and politicians) that it is the only legitimate healing practice, scientific medicine's cultural authority (as described in Chapter 2) is protected. This in turn restricts competition for patients and for private and public money spent on health care. How has this been done?

One way to do so was through an educational campaign, using the vast public relations resources of the AMA and other organizations to expose the dangers and errors of these cults. Another approach was to employ political leverage and legal muscle. Organized medicine excluded from its ranks those who espoused such systems; denied such practitioners the privilege of consultation; refused to see patients when such healers were assisting in the case; prevented such practitioners from working in or otherwise using public hospitals; went to court to prosecute them for violating existing medical practice acts; and actively opposed legislative protection for them or, when that failed, opposed allowing them any additional privileges. (Gevitz, 1988:16–17)

CAM's View of Conventional Healers

Practitioners and proponents of complementary and alternative healing practices view their work in a completely different way. Many have argued that their goal is the same as that of conventional medicine: to offer effective healing therapies. Their belief is that orthodox medicine has helped some people but has failed to help many others and, in fact, often harms them (e.g., negative drug reactions or drug dependency).

CAM healers contend that the many people who have been helped by their practices, the high levels of satisfaction in their patients, and the high percentage of people who see them on a continuing basis testify to the efficacy of their treatments. They believe patients should have an unencumbered right to choose their healing practice from a variety of options, just as they have a right to choose their religion. If a particular type of healing practice is worthless, patients will soon discover that, and the demand for that service will diminish. Alternative healers have often asked for the right to practice without attack from organized medicine. See the box, "The Ability of Teenagers to Choose a CAM."



IN THE FIELD

THE ABILITY OF TEENAGERS TO CHOOSE A CAM

In 2005, a 15-year-old Virginia teenager, Abraham Cherrix, underwent three months of chemotherapy for Hodgkin's disease, a type of cancer. The treatment left him so weak and so nauseated that at times he had to be carried by his father because he couldn't walk. In February 2006, when he learned that the cancer had become active once again, he refused to undergo another round of chemotherapy and radiation. He said that he did not think that he could live through it. Hodgkin's is generally considered a treatable condition and has a five-year survival rate of about 80 percent. However, with the reoccurrence, Abraham's survival possibility was estimated at 50 percent.

Instead, after doing significant reading, he chose a sugar-free organic diet that included

lots of fruits and vegetables, herbs, and visits to a clinic in Mexico. This raised the issue of the age at which an individual is able to make lawful decisions for himself or herself. His parents supported him in his choice, saying that they believed he is a mature and thoughtful young man. In May, a judge issued a temporary order finding Abraham's parents neglectful for supporting his choice. They were ordered to give partial custody of Abraham to the County Department of Social Services, so that they could have the chemotherapy and radiation continued. The parents were told that if they refused to comply that they would lose all custody of Abraham.

In August 2006, a Circuit Court judge cleared the family of all charges of medical neglect and allowed them to follow their treatment course

of choice as long as Abraham would be periodically examined by a specific board-certified oncologist in Mississippi who is experienced in alternative cancer treatments. This resolution was acceptable to Abraham and his parents. On his 18th birthday in 2008, Abraham

was doing well, but the Hodgkin's returned in 2009. In the succeeding years, Abraham has had recurrent bouts of Hodgkin's separated by times in which he has felt well. At last report, Abraham was being treated with infrared saunas, herbs, and nutritional supplements.



The case of Abraham Cherrix—shown here—raised the important question about whether mature individuals under the age of consent may choose to follow CAM rather than conventional medicine.

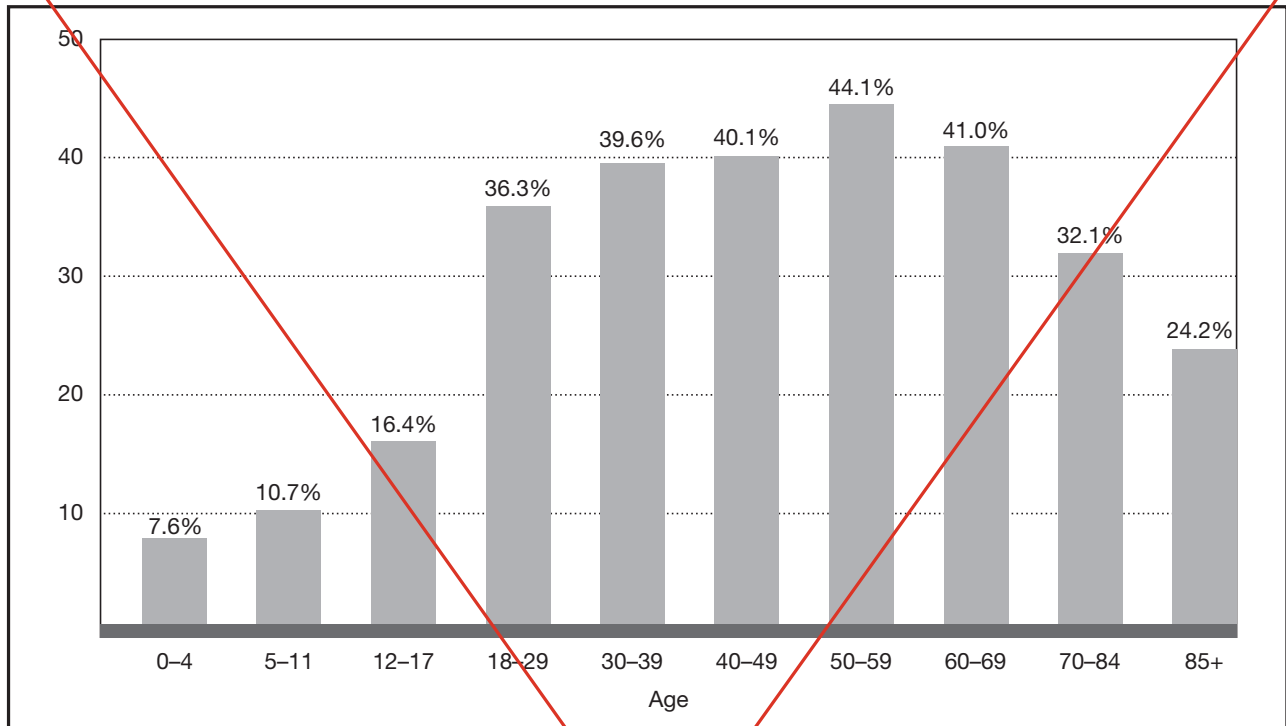
COMPLEMENTARY AND ALTERNATIVE HEALERS

Use of Complementary and Alternative Healers

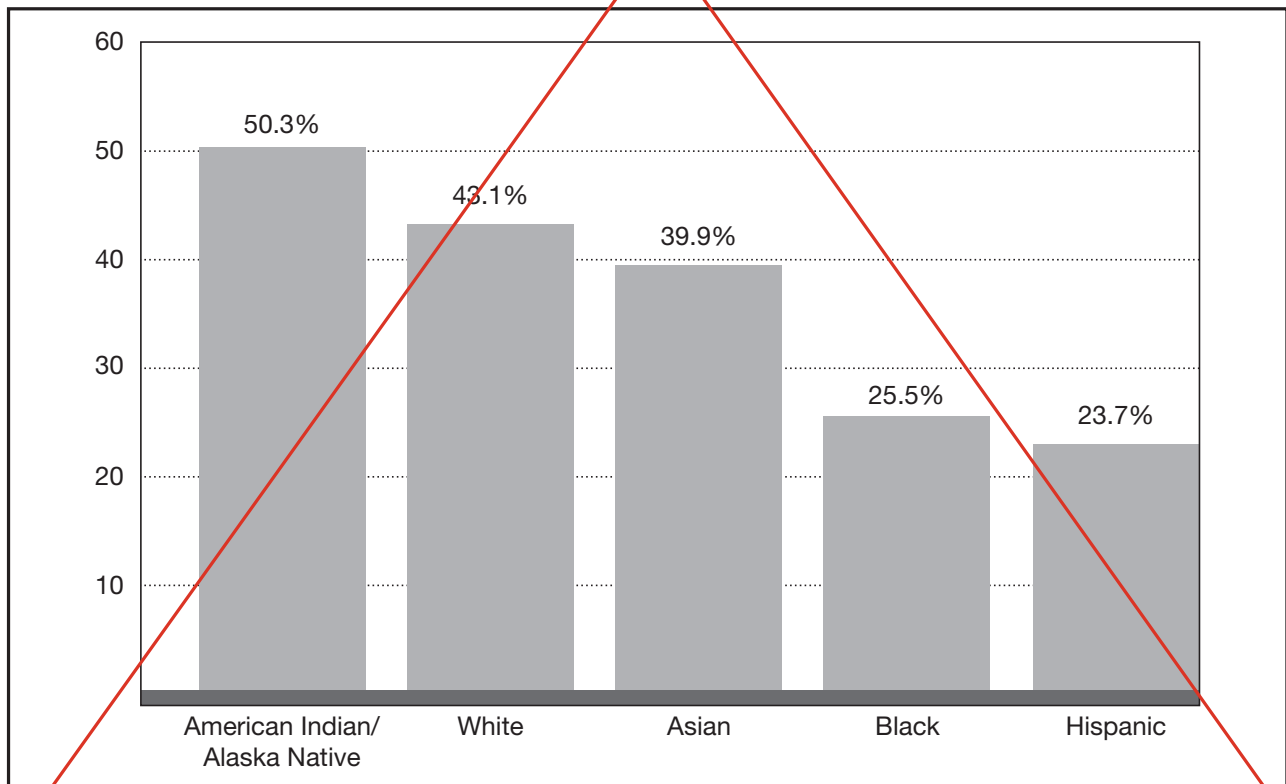
It is now recognized that millions of people use complementary and alternative healers every year. About half of American adults use at least one form of alternative medicine, and 10 percent use it on their children. Annually, Americans pay more visits to CAM healers than to primary care physicians. When Oxford Health Plans included use of CAM in its benefit package, between 40 and 50 percent of members saw a CAM provider in the first year (Kilgore, 1998). Many health maintenance organizations around the country have now begun including coverage for CAM. CAM has become a \$34 billion a year industry.

Use of CAM is common by both men and women (females slightly more likely), by both blacks and whites (whites somewhat more likely), among people of all ages (with especially high use among middle-aged persons), among people at all levels of education (with use highest among the most well educated), and in all socioeconomic groups (with use highest among those with the most income) (Barnes, Bloom, and Nahin, 2008; Grzywacz et al., 2007). Research has also shown that many people who cannot financially access the health care system often turn to CAM because it is more affordable (Pagan and Pauly, 2005). Figures 11-1 and 11-2 report additional patterns of use of CAM services in the United States.

The most common problems for which CAM therapies are used are back pain, neck pain, and

Figure 11–1 CAM Use by Age, 2007

Source: Patricia M. Barnes, Barbara Bloom, and Richard L. Nahin, "Complementary and Alternative Medicine Use Among Adults and Children: United States, 2007," *National Health Statistics Report*, Number 12 (Hyattsville, MD: National Center for Health Statistics, 2008).

Figure 11–2 CAM Use by Race/Ethnicity Among Adults, 2007

Source: Patricia M. Barnes, Barbara Bloom, and Richard L. Nahin, "Complementary and Alternative Medicine Use Among Adults and Children: United States, 2007," *National Health Statistics Report*, Number 12 (Hyattsville, MD: National Center for Health Statistics, 2008).

joint pain. Table 11–1 identifies the ten most frequently cited reasons for using a CAM practitioner. The most commonly used CAM therapies are reported in Table 11–2, and the most commonly used natural products are found in Table 11–3.

The Dual Model of Care

Are all or most of the people who use CAM completely dissatisfied with conventional medical care? No. Researchers have discovered that many people follow a “dual model of medical care,” making use of an alternative healer at the same time as they receive care from a medical doctor.

While some people have become disillusioned with conventional care and have made a cognitive commitment to complementary and alternative practices, the more common pattern is that individuals use different healers for different problems. For example, many patients consult with chiropractors about chronic low back pain but continue to rely on medical doctors for other problems. Their selection of a healer is made on very pragmatic grounds: They continue to see medical doctors for most ailments because that has been helpful in the past, but

TABLE 11–2 Ten Most Common CAM Therapies Used by Adults, 2007

Therapy	Used in the Last Year (%)
Natural products	17.7
Deep breathing	12.7
Meditation	9.4
Chiropractic/Osteopathic	8.6
Massage	8.3
Yoga	6.1
Diet-based therapies	3.6
Progressive relaxation	2.9
Guided imagery	2.2
Homeopathic treatment	1.8

Source: Patricia M. Barnes, Barbara Bloom, and Richard L. Nahin, “Complementary and Alternative Medicine Use Among Adults and Children: United States, 2007,” *National Health Statistics Report*, Number 12 (Hyattsville, MD: National Center for Health Statistics, 2008).

if their back pain has received little relief from the family doctor, they will seek relief from a chiropractor. If that works, they will maintain allegiance to both practitioners—each in a specified domain (Kelner and Wellman, 1997; Kronenfeld and Wasner, 1982). In a 1985 study of asthma patients using an alternative healer,

TABLE 11–1 Ten Most Common Reasons for Adults to See a CAM Practitioner

Condition	Used Alternative Healer in the Last Year (%)
Back pain	17.1
Neck pain	5.9
Joint pain	5.2
Arthritis	3.5
Anxiety	2.8
Cholesterol	2.1
Head or chest cold	2.0
Other musculoskeletal pain	1.8
Severe headache/Migraine	1.6
Insomnia	1.4

Source: Patricia M. Barnes, Barbara Bloom, and Richard L. Nahin, “Complementary and Alternative Medicine Use Among Adults and Children: United States, 2007,” *National Health Statistics Report*, Number 12 (Hyattsville, MD: National Center for Health Statistics, 2008).

TABLE 11–3 Ten Most Common Natural Products Used by Adults, 2007

Therapy	Used in the Last Year (Of Those Using a Natural Product) (%)
Fish oil/Omega 3	37.4
Glucosamine	19.9
Echinacea	19.8
Flaxseed oil pills	15.9
Ginseng	14.1
Combination herb pills	13.0
Ginkgo biloba	11.3
Chondroitin	11.2
Garlic supplements	11.0
Coenzyme Q-10	8.7

Source: Patricia M. Barnes, Barbara Bloom, and Richard L. Nahin, “Complementary and Alternative Medicine Use Among Adults and Children: United States, 2007,” *National Health Statistics Report*, Number 12 (Hyattsville, MD: National Center for Health Statistics, 2008).

more than three-fourths reported that they were satisfied with *both* their medical doctor and the alternative healer (Donnelly, Spykerboer, and Thong, 1985). However, several studies have discovered that most persons who follow the dual model of care do not inform their medical doctor that they are also seeing a CAM healer, even if it is for the same complaint. One survey found that almost three-fourths of Americans age 50 and older have not talked to physicians about nontraditional treatments being received (National Center for Complementary and Alternative Medicine, 2013).

The Efficacy of Complementary and Alternative Healers

Is this information a valid and reliable indication that at least some CAM healers offer efficacious treatment? Possibly, but not necessarily. Determining the efficacy of any medical treatment, conventional or unconventional, is more complicated than it might seem at first. Rodney Coe (1970) identified three reasons why the use of magic in primitive medicine is (or seems to be) effective. These reasons can be generalized to any form of medical treatment whatever the level of scientific sophistication in the society.

First, in all societies, most patients most of the time will recover regardless of the form of treatment received or even whether any treatment is provided. The amazing recuperative powers of the human body are only now being recognized. The same point is made by the old adage about seeing a physician for a cold: If you do, you'll be well in a week; if you don't, it will take seven days. Thus, whether one receives muscle relaxants from a medical doctor or spinal manipulation from a chiropractor, one's back pain will usually diminish eventually. Typically, we give credit to whatever treatment was received, although we would often have healed without treatment.

Second, when patients believe strongly in the medical care they receive, it has great psychotherapeutic value, whatever its direct effects. The determination to get well and the confidence that recovery will occur are relevant factors in the

healing process. Believing in the cure offered by your family physician can contribute to its success, just as believing in the efficacy of being needled by an acupuncturist or sharing prayer with a Christian Science practitioner can.

Finally, some medical practices are empirically correct, even though there is not a clear explanation. Coe uses the example of a medicine man treating a snakebite victim. He might open the wound further and suck out the evil spirit that had entered. In so doing, he is actually sucking out the poisonous venom from the wound, thus accomplishing what orthodox medicine would recommend but basing it on an entirely different underlying theory.

Furthermore, every medical treatment must be considered within the context of the practitioner–patient relationship. It is now well recognized that the quality of this relationship may influence the course of treatment and the healing process.

Treatments offered by alternative healers are often enhanced by the greater rapport they develop with patients. Alternative healers are often more sympathetic than medical doctors to minor but nagging conditions that can trouble an individual. Most of the alternative healing practices involve more talking and more touching, both tremendously reassuring processes, than are often involved in treatment by the medical doctor. Alternative healers are also viewed as giving more time (more than four times as much time as M.D.s give per patient); doing a better job of avoiding medical jargon; and providing warmer, more relaxed treatment settings. One study found that individuals who use CAM are more likely than those who do not to report their own health as being excellent and their health to have improved in the last year (although the study was not done in such a way as to determine causality) (Nguyen et al., 2011).

All this is not to say that we do not make individual judgments about the efficacy of medical care received. We do. And it is not to say that patterns of efficacy cannot be studied. They can. But it is to say that drawing firm conclusions about the efficacy of any form of medical care must be done very carefully.

Mainstream Interest in Complementary and Alternative Healing Practices

In recent years, interest in complementary and alternative healing practices has increased among policy makers. In 1992, the U.S. Senate established the Office of Alternative Medicine (OAM) within the National Institutes of Health to evaluate the effectiveness of unconventional medical practices. Now called the National Center for Complementary and Alternative Medicine (NCCAM), the first-year budget of \$2 million has grown more than 60-fold to over \$128 million in 2012. Much of the money has been allocated to research projects on various practices and to public education.

Many medical schools have started or are developing courses on alternative therapies—more than three-fourths of medical schools in the United States now offer at least one CAM course. Many hospitals now offer selected CAM techniques such as massage therapy and acupuncture. Health insurance policies now routinely cover care from at least some types of alternative healers, and Washington has become the first state to pass an “any-willing-provider” law that requires insurers to cover the services of every licensed or certified health care provider in the state.

The general attitude of medical doctors toward CAM practices has certainly softened. While some remain skeptical of approaches that have not undergone rigorous scientific testing, many physicians now accept the value of at least some of the practices and routinely refer patients who they are unable to help to CAM practitioners. Efforts are also underway to better disseminate to physicians the results of clinical trials of CAM healing practices (Tilburt et al., 2009).

In this chapter, we examine four CAM practices: chiropractic, acupuncture, religious healing (particularly Christian Science), and ethnic folk healing (curanderismos and Navajo healers).

CHIROPRACTIC

The field of **chiropractic** contains many contradictions. Millions of people in the United States enthusiastically support chiropractic, while

many continue to see it as nothing more than successful quackery. (A former president of the American Chiropractic Association was fond of saying, “People either swear by us or at us.”) Without altering its basic philosophy or practice, it has achieved increased acceptance by many M.D.s, yet is condemned by others. There is even dissensus among chiropractors themselves as to the appropriate boundaries of the field.

Nevertheless, certain facts are clear. Chiropractic is a licensed health profession in all 50 states, and chiropractors are recognized and reimbursed by federal, state, and most commercial insurance companies. There are more than 60,000 licensed doctors of chiropractic in the United States, and an estimated 20 million people are treated by chiropractors each year. It represents an important component of the health care system.

Origin

The field of chiropractic was founded by Canadian-born Daniel David Palmer (1845–1913), a healer living in Davenport, Iowa. Palmer credited his vision of the field to two successful experiences he had in 1895: By realigning displaced vertebrae, he restored hearing to one man who had become deaf 17 years earlier when something had “given way” in his back and he relieved another patient’s heart problems. He reasoned that if two such disparate conditions could be treated through manipulation of the vertebrae, potential existed for curing all ailments in this fashion. His research findings, published in 1910, served as the foundation for this new healing practice and the basis for a chiropractic school, which he established in 1897 (Wardwell, 1992).

Basic Principles

The National Center for Complementary and Alternative Medicine (2010b) identifies the three basic tenets of chiropractic:

1. The body has a powerful self-healing ability. Illness results from a failure to maintain homeostasis—the positive bodily drive

toward health. Maintenance of homeostasis occurs through good nutrition, good posture, exercise, stress management, creative meditation, and natural (nonpharmacological) healing. Serious disease is viewed as the end result of a process that could have been avoided through this holistic approach.

2. The body's structure (primarily that of the spine) and its function are closely related, and this relationship affects health. Vital energy flows throughout the body's nerves during a homeostatic state. However, this energy can be blocked by subtle malalignments of the vertebrae called subluxations. These subluxations are the origin of most human illness.
3. Therapy aims to normalize this relationship between structure and function and assists the body as it heals. By correcting the



Chiropractors believe that health problems are expressions of underlying problems including blockages of the flow of vital energy caused by malalignments of the vertebrae.

spinal malfunction, the chiropractor expects the specific problem to disappear and the patient's general health to improve.

Caplan (1984) explicates the three main ways that this philosophy of healing contrasts with orthodox medicine:

1. Medical doctors typically equate symptoms with particular diseases and identify health as the absence of symptoms. Disease is discovered when symptoms appear and is usually judged to have ended when the symptoms disappear. Chiropractic does not make this equation. A body is assumed to be diseased for some period of time before symptoms appear. Rather than focusing on symptoms, chiropractors focus on the subluxation.
2. Orthodox medicine considers germs to be the underlying cause of many disorders. The medical profession considers this to have been scientifically proven and disdains any healing philosophy that does not subscribe to it. The task of the physician is to diagnose what germs are present and offer treatment to reduce their presence in order to restore health.
Chiropractic's adherence to the belief that subluxations cause disease is in seeming contradiction to scientific medicine. Chiropractors believe that germs are a necessary condition for many diseases, but not a sufficient one. For a disease to occur, the host must have been made susceptible to the disease by factors such as poor nutrition, stress, heredity, and vertebral subluxations.
3. While chiropractors see themselves as holistic healers specializing in preventive care, some physicians believe the field should be restricted to musculoskeletal conditions or eliminated altogether.

Historical Developments

Early on, the field of chiropractic split into camps. The first camp maintained that chiropractic was not the practice of medicine and ought to offer solely spinal adjustment as a therapeutic modality. An alternative view

developed among many of the M.D.s who were also doctors of chiropractic that chiropractors ought to offer a wide variety of treatment techniques in addition to spinal adjustment. The former school became known as “straights,” while the latter group became known as “mixers.” Eventually, each group began its own national association: the American Chiropractic Association (ACA) for the straights and the International Chiropractors Association (ICA) for the mixers. Even at this time, however, most viewed the two associations as having much more in common than in conflict.

The field of chiropractic struggled through its early years. The Flexner Report of 1910 condemned existing chiropractic schools for failure to develop ties with universities and for the absence of demanding training programs. The Great Depression significantly reduced philanthropic contributions and cut back applications for schooling.

However, the most important early battle for chiropractic was to gain state-sanctioned licensure. It was not an easy battle as chiropractors were often jailed for practicing medicine without a license. Ultimately, being jailed became a successful strategy used to win public support for the field as a reaction to its persecution. Kansas passed the first chiropractic licensing law in 1913; 39 states gave some form of legal recognition by 1931; and, in 1974, when Louisiana began licensure for chiropractors, acceptance had been won in every state.

Wardwell (1992) has suggested that 1974 was the turning point for chiropractic. In addition to Louisiana's accepting licensure, the U.S. Office of Education gave the Chiropractic Commission on Education the right to accredit schools of chiropractic; the federal government determined that chiropractors' fees were reimbursable under Medicare; and Congress authorized spending \$2 million for a study of the merits and efficacy of chiropractic treatment by the National Institute of Neurological Disorders and Stroke (NINDS) of the National Institutes of Health. The NINDS Conference concluded that spinal manipulation does provide relief from pain, particularly back pain, and sometimes even cures.

Organized Medicine and Chiropractic

For decades, the American Medical Association (AMA) attempted to drive chiropractic out of existence. As early as 1922, AMA officials adopted the slogan, “Chiropractic must die” (Reed, 1932). In 1963, the AMA's Committee on Quackery referred to the elimination of chiropractic as its ultimate mission and engaged in such activities as “producing and distributing anti-chiropractic literature, communicating with medical boards and other medical associations, fighting chiropractic-sponsored legislation, and seeking to discourage colleges, universities, and faculty members from cooperating with chiropractic schools” (Gevitz, 1989:292).

In 1965, the AMA declared it a violation of medical ethics for M.D.s to have any professional association with chiropractors. Presumably, this proscription included making or accepting referrals of patients; providing diagnostic, laboratory, or radiology services; teaching in chiropractic schools; and practicing jointly in any form.

What was the motivation for such strong action? Organized medicine defended its actions on the ground that it was attempting to eliminate a practice it considered to be detrimental to patient welfare. Believing the vertebral subluxation concept to be grossly inaccurate and denigrating the low standards of chiropractic education, orthodox medicine defined chiropractic as having little or no therapeutic value and as being potentially dangerous. The AMA viewed these efforts as being consistent with its responsibility for guarding the public against medical quacks and charlatans.

On the other hand, chiropractic contended that the AMA's actions were motivated by professional elitism (not wanting to share the prestige of the medical profession) and an effort to restrict economic competition. In fact, these contentions became part of a lawsuit brought by chiropractors in 1976 against the AMA for violating the Sherman Antitrust Act.

Whether motivated by sincere change in ideology or fear of an expensive defeat in the courts, the AMA instituted changes in policy during the decade-plus that the case was bogged

down in court. In 1978, the AMA adopted the position that medical doctors could accept referrals from and make referrals to chiropractors; and in 1980, the Principles of Medical Ethics were revised to eliminate the professional association prohibition.

Nevertheless, in August 1987, U.S. District Judge Susan Getzendanner found the AMA, the American College of Radiology, and the American College of Surgeons guilty of violating the Sherman Antitrust Act. She judged that the three associations had acted conspiratorially in instituting a boycott of chiropractic and had failed to justify it by its "patient care defense." She required that the actions cease.

Current and Future Status

Chiropractic clearly has established a very important place in the health care system. The educational preparation for chiropractors has continued to be upgraded. The curriculum is now comparable to that in medical schools with respect to study of the basic sciences; the major difference is that chiropractic students take courses in spinal analysis and manipulation and nutrition rather than in surgery and pharmacology (chiropractors are prohibited from doing surgery or prescribing drugs). Chiropractors must pass a national examination administered by the National Board of Chiropractic Examiners and must be licensed by a state board. Unlike many medical doctors, chiropractors are required to continue their education in order to retain their license (Wardwell, 1992). Both the ACA (with 9,000 members) and the ICA (with 8,000 members) promote the field of chiropractic and view the field as being patient oriented and wellness focused.

Unquestionably, many people believe strongly in the value of chiropractic treatment. Several studies have reported benefits of chiropractic care. A 1982 New York study found that almost three in ten (28 percent) persons had been examined by a chiropractor at some time, that 72 percent of recent users found it to be very effective, and that 92 percent would definitely or probably see a chiropractor again should a

need arise. Numerous other studies confirm that an increasing percentage and genuine cross section of the population visits chiropractors, that patient satisfaction is quite high, and that the general prestige of the field is on the upswing (Meeker and Haldeman, 2002).

Has all this changed the attitudes of physicians toward chiropractic? Apparently, it has to some extent. An increased number of medical doctors now recognize the benefit that chiropractic has for some patients, and medical doctors and chiropractors are increasingly likely to make and accept referrals from each other.

What will be the future status of chiropractic in American society? Aside from a continuation of the status quo, Wardwell (1988) suggests four possibilities:

1. Chiropractic could be absorbed by orthodox medicine as a routine part of medical practice, and it could be taught to all medical doctors (especially those in appropriate specialties) as part of their medical education. While chiropractors fear this eventuality, it is not likely that orthodox medicine could easily absorb the practice or that there would be sufficient M.D.s to provide all the required treatments.
2. Chiropractic could become a profession but be subordinate to physician supervision. While organized medicine may prefer that manipulative therapy be done by physical therapists (or chiropractors serving as physical therapists), this option is not appealing to chiropractors who would have to surrender the autonomy of having patients come directly to them.
3. Chiropractic could practice in a limited domain but be independent of supervision or regulation by organized medicine. While achieving this status would require chiropractic to modify its underlying theory, Wardwell believes that this is possible and is the most likely future direction for the field.
4. Chiropractic could become a parallel profession (like osteopathic medicine) by elevating its standards of medical training, reducing the gulf in underlying theory, and continuing to gain greater acceptance in the eyes of the

public and organized medicine. Clearly, many young chiropractors prefer this possibility and would like to be seen as appropriate family care providers. However, some skepticism in organized medicine remains, and many chiropractors are determined not to compromise their basic practice modality. Nevertheless, this seems to be the current direction.

ACUPUNCTURE

Chinese understanding of health and illness has evolved over nearly 3,000 years and is recorded in more than 6,000 texts. Traditional Chinese medicine is a holistic system in which health is understood only in the context of the relationship between the human body and nature.

Medical theory rests on the belief that each object in nature is both a unified whole and a whole composed of two parts with opposing qualities: **yin and yang**. They are constantly in a dynamic interplay, shifting from being opposites to becoming each other. Yin represents the cold, slow, and passive principle, while yang represents the hot, excited, and active principle. Health is achieved and maintained by maintaining a balanced state within the body; disease occurs when the yin and yang forces become imbalanced (National Center for Complementary and Alternative Medicine, 2013).

An imbalance leads to a blockage that disrupts the flow of chi (vital energy) within the body. Chi is considered to be the primary force of nourishment and bodily protection. Though there is nothing exactly comparable in Western thought, it is sometimes viewed as the “will to live.” Chi flows through the body through 12 (or 14 or 20; sources vary) main channels (or meridians), activating energy in the circulatory system as it flows. The exact location of these channels has been charted and diagrammed, and each is thought to represent (and be connected with) an internal organ. Some of the exercises and martial arts performed by Chinese people stimulate the flow of this vital energy in the channels.

The harmony and balance within the body may be disrupted either by endogenous factors,

which originate from some serious internal imbalance, or by exogenous factors, which come from the external environment and may be physical (e.g., climactic conditions) or biological (e.g., bacteria and viruses). In performing diagnosis, traditional Chinese medicine follows the principle, “anything inside is bound to manifest outwardly.” An implication of this principle is that even localized symptoms (e.g., a headache) are not viewed as local disturbances, but rather as a sign of abnormality within bodily organs and the body’s channel system. Therefore, a headache does not necessarily mean an imbalance in or near the head.

The primary goal of Chinese medicine is to restore the internal balance of the body and the harmony between the environment and the human being. Since the body’s internal balance is constantly fluctuating, specific treatment must be tailored to the situation at the time.

While much of Western attention has focused on acupuncture as a treatment technique, it is actually only one of many options in traditional Chinese medicine. Other important treatment techniques include acupressure (significant pressure applied to the body via the fingertips), herbology (the use of natural herbs), moxibustion (placing ignited moxa wool on certain points of the body to create heat), various breathing exercises, physical activity, massage, and cupping (placing a small jar with a partial vacuum created by a flame over a selected part of the body producing an inflammatory response). Due to acupuncture’s unique history in the United States, this section will focus on it as a healing practice.

Origin of Acupuncture in the United States

Even though it has been used in China for almost 3,000 years and was often practiced by Chinese immigrants in the states, acupuncture gained broad, popular attention in the United States only in the early 1970s. This discovery of acupuncture can be traced to two events that occurred in 1971: the lifting of the “bamboo curtain” with China, which opened relations between the countries, and an attack of

appendicitis suffered by famed *New York Times* columnist James Reston while he was visiting China—acupuncture was used the day after surgery to eliminate significant pain. Reston wrote of his experience in the *Times*, thus drawing widespread interest to the subject. A select group of American physicians (including a delegation from the AMA) visited China in the ensuing months, and their glowing reports of the efficacy of acupuncture ensured further popular and professional attention (Wolpe, 1985).

Basic Principles

Acupuncture is the insertion of fine needles into one or more acupuncture “points” charted on the body. Today, there are more than 700 points that have been identified, although only 40 or 50 are commonly used. The needles used vary in length, width, and type of metal, and treatments vary in the depth to which needles are inserted, the duration of insertion, and the needle rotation.

The insertion of the needles is performed to stimulate chi in the body and to redirect it so that imbalances are corrected. The needles are inserted in those points that correspond to the particular internal organs where the imbalance exists.

Historical Developments

Although American physicians largely focused on the anesthetic value of acupuncture, and ignored its therapeutic utility, the popular press offered vivid descriptions of acupuncture as a miracle process. The federal government encouraged research into acupuncture, and scientific journals published scores of articles. The Internal Revenue Service decided that payments for acupuncture service qualified as a medical expense, and the Food and Drug Administration developed quality control regulations for acupuncture needles. An American Society of Chinese Medicine was formed (Wolpe, 1985).

In July 1972, the first acupuncture clinic opened in New York City. When it was shut down a week later for practicing medicine without a license, it had already served 500 patients

and was booked solid for several months. Acupuncture had captured America by storm. Although the manner in which it would be incorporated remained to be determined, acupuncture seemed on the verge of becoming a major healing practice in the United States.

Organized Medicine and Acupuncture

However, the medical establishment quickly reigned in the enthusiasm. One can understand the professional embarrassment created by a healing practice based on a theory that seemed completely contradictory to “scientific” medicine. The tremendous media and lay interest in acupuncture

quickly became anathema to most of organized medicine. Physicians had no expertise in acupuncture and no knowledge of physiological mechanisms that could account for it. Indeed, it seemed to violate laws of anatomy and neurophysiology. Acupuncture was an alien treatment with an alien philosophical basis imported as a package from the East; it was not an indigenous alternative modality that reacted to (and thus was informed by) the biomedical model. (Wolpe, 1985:413)

Negative and often hostile physician reactions escalated. The therapeutic effects of acupuncture were dismissed as being nothing more than placebo, and its effectiveness as an anesthetic was dismissed as being a type of hypnosis or form of suggestibility, or even that “Chinese stoicism” or patriotic zeal enabled patients to undergo excruciatingly painful surgery without other anesthetics (Wolpe, 1985). One physician referred to acupuncturists as “nonscientific weirdoes” and attempted to portray the entire acupuncture practice as modern-day quackery (Goldstein, 1972).

Wolpe (1985) contends that in order to protect its cultural authority over medicine, the medical establishment employed two additional strategies. The first was to sponsor and conduct research that would explain acupuncture in terms of the traditional biomedical model. While Chinese practitioners strongly believe that the practice cannot be separated from its underlying theory, and therefore cannot be studied by traditional scientific methods, much research has

been conducted to find a conventional explanation for its anesthetic effects.

For example, Melzack and Wall (1965) developed the “gate control theory” based on research that shows that the insertion of needles excites certain nerve fibers that enter the spinal column and inhibit the onward transmission of pain to the brain. Stimulating these nerve fibers effectively “closes the gate” to pain. An alternative theory, also consistent with traditional neurophysiology, is that the needle insertion stimulates the release of certain pain-reducing hormones (endorphins and enkephalins), which create the anesthetic or analgesic effects. Some recent research has found that needle insertion reduces the flow of blood to the areas of the brain that control pain.

The second strategy was to arrange for the practice of acupuncture to be placed under the jurisdiction of medical doctors. After all, if acupuncture was beneficial only for pain relief, and if the explanation for that could be provided in conventional terms, then it could be argued that Oriental practitioners were not as able as Western practitioners to provide safe and effective treatment. Regulations governing the practice of acupuncture were quickly established in many states (either by the legislature or the State Medical Board).

Current and Future Status

Today, there are 60 accredited schools of Oriental Medicine in the country, and more than 20,000 licensed (by the National Certification Commission for Acupuncture and Oriental Medicine or NCCAOM) practitioners. The NCCAOM offers certification in Oriental Medicine, Acupuncture, Chinese Herbology, and Asian Bodywork Therapy. Regulation is provided by states, but with different requirements. Most states require a practitioner to have a Master of Acupuncture or Oriental Medicine degree or its equivalent and national certification. It is estimated that more than 3 million adults in the United States use acupuncture each year. Many private insurers and the Medicaid programs in some states now cover the cost of acupuncture treatment.

Is acupuncture an effective anesthetic or therapeutic healing practice? Considerable research has reported favorable findings. In late 1997, a panel of scientists at the National Institutes of Health (including some who practice acupuncture and some skeptics of it) concluded that acupuncture clearly works in treating many conditions including nausea and vomiting after chemotherapy and surgery, the



Most states require traditional Chinese medicine practitioners to have a Master of Acupuncture or Oriental Medicine degree or its equivalent and national certification.

nausea of pregnancy, and postoperative dental pain. Although less data were available, they concluded that acupuncture may help stroke rehabilitation and relieve addictions, headaches, menstrual cramps, a variety of muscle pains, carpal tunnel syndrome, insomnia, arthritis, and asthma. They acknowledged that these benefits occur while acupuncture has fewer side

effects and is less invasive. The World Health Organization now recognizes more than 40 conditions as being treatable by acupuncture. Acupuncture is a fully accepted and practiced therapy in countries around the world.

The accompanying box, “Legalizing Medical Marijuana,” discusses a CAM technique that is currently in the news.



IN THE FIELD

LEGALIZING MEDICAL MARIJUANA?

In 1996, California voters approved Proposition 215, which permitted physicians to *recommend* marijuana for their patients. Because it would violate federal law, physicians were prohibited from *prescribing* it. In 1997, the *New England Journal of Medicine* endorsed the legalization of medical marijuana, arguing that it has clearly brought relief from pain for many people. Later that year, the AMA, though rejecting endorsement of legalization, called for the right of physicians to discuss any treatment alternatives with patients without possibility of criminal sanction. By 2012, 16 additional states (Arizona, Alaska, Colorado, Connecticut, Delaware, Hawaii, Maine, Michigan, Montana, Nevada, New Jersey, New Mexico, Oregon, Rhode Island, Vermont, and Washington) allowed cannabis to be cultivated and used for medical purposes, and several now allow retail pot dispensaries (although some of the dispensaries have been accused of selling marijuana to anyone). Oregon permits medical marijuana cardholders to socialize at designated cafes and to use free, over-the-counter cannabis. Many more states are considering similar legislation. In 2012, Colorado and Washington voted to legalize recreational use of marijuana.

Until 2009, the U.S. Justice Department maintained that federal law prohibits any use of marijuana and indicated that it would use its authority under the Controlled Substances Act to revoke the license to prescribe drugs of any physician who *recommended* marijuana to a patient. In May 2001, the U.S. Supreme Court ruled that the federal law prohibiting the manufacture and distribution of marijuana means that it cannot be sold or used for medicinal

purposes. Although medical marijuana users are typically not prosecuted, they are in the awkward position of engaging in a behavior that has been expressly approved by their state but disapproved by the federal government. In 2009, the Attorney General of the United States, Eric Holder, indicated that President Obama would follow his campaign promise to stop raids on state-approved marijuana dispensaries (Hoffman and Weber, 2010).

Can marijuana be medicinal? Yes. Research has shown four beneficial medical effects: (1) It reduces the nausea associated with cancer chemotherapy; (2) it reduces “wasting syndrome”—the deadly loss of appetite and consequent weight loss that many AIDS patients feel near the end of life; (3) it reduces the painful muscle spasms and tremors experienced by many people with spinal cord injuries and multiple sclerosis; and (4) it reduces pressure inside the eye for persons with glaucoma (although another drug is now more effective). There is experimental evidence that it may be helpful in treating other conditions such as depression, Crohn’s disease, hepatitis C, and multiple sclerosis.

Are there any demonstrated negative side effects of marijuana use? Yes. Two such effects are: (1) it impairs cognitive functioning, negatively affecting coordination and short-term memory (studies have been inconsistent on whether there is any long-term cognitive impairment), and (2) it leads to respiratory damage (studies have found that smoking marijuana is even harder on the lungs than smoking tobacco).

Proponents of medical marijuana argue that individuals should have the right to decide

for themselves whether the benefits outweigh the harms. Because many of the users are and would be light users and would be using it only on a temporary basis, they argue that the harms are overstated. Besides, it is well known that many persons with cancer and with AIDS have long been using marijuana for relief but have been forced to do so surreptitiously. Opponents argue that liberalizing

use of the drug might lead to increased use of marijuana and/or other drugs and that the harms of the drug justify its continued ban. In addition, pharmaceutical companies, anticipating the possibility of patenting (potentially very profitable) drugs that include tetrahydrocannabinol (THC)—the active ingredient in marijuana—have opposed legalization of medical marijuana.

SPIRITUAL HEALING AND CHRISTIAN SCIENCE

A belief in “psychic healing” has been present in early and modern times and in Western and non-Western cultures. Psychic healing “refers to the beneficial influence of a person on another living thing by mechanisms which are beyond those recognized by conventional medicine. These mechanisms may include focused wishes, meditation, prayers, ritual practices, and the laying-on-of-hands” (Benor, 1984:166).

Psychic healers use one of four approaches: (1) activating innate recuperative forces within the patient; (2) transferring her or his own healing energy to the patient; (3) serving as a conduit through which universally available cosmic energy is transferred to the patient; or (4) serving as a conduit through which the healing powers of spirits or God are transferred to the patient. The final channel is also referred to as **spiritual healing** or “faith healing” (Benor, 1984). Spiritual healers do not claim any personal ability to heal but rather an ability to convey the power of some transcendent being to the sick. Spiritual healers may or may not be affiliated with a particular church and may or may not be full-time healers.

Efficacy of Spiritual Healing

Determining the efficacy of spiritual healing is difficult. Most of the evidence supporting positive effects is anecdotal and comes from people strongly predisposed to its benefits. Other more systematic research has been done but in ways that allow alternative explanations of the findings.

Many people accept that there are at least some cases where a subject’s health status has improved following a spiritual healing encounter. These cases are interpreted in different ways. Those involved in the healing process typically contend that God has intervened and in a miraculous way effected a cure. Others believe that the health improvement or cure has occurred through psychological processes, for example, marshaling the patient’s mental powers and determination to combat the ailment and/or convincing the patient that a cure will occur (akin to the placebo effect).

In a study of spiritual healing groups in Baltimore in the early 1980s, Glik (1988) discovered that most of the reported illnesses were chronic and mild to moderate in severity, with “nonspecific” diagnosis (but related to psychosomatic, stress, or mental health problems). Less than 10 percent of the reported conditions were life threatening or serious. Many of the symbols and rituals used in the groups (as well as the supportive social context) would be likely to affect the psyche of those present.

In the largest study ever done on the topic, researchers from six hospitals throughout the United States examined the effects of intercessory prayer, that is, someone praying for another for therapeutic improvement (Benson et al., 2006). Previous well-controlled clinical tests had not supported such benefits, but they had not addressed whether any outcomes were due to prayer itself or to the knowledge that prayer was being offered by someone. Patients in the study were divided into three groups: (1) 604 individuals who received prayer after being informed they may or may not receive it,

CHAPTER 12

The Physician–Patient Relationship: Background and Models

Learning Objectives

- Identify and explain each of the three key dimensions of the physician–patient relationship.
- Discuss the extent to which patients want to be fully informed about their health and be active participants in their health care. Identify and discuss key reasons that patient expectations are often not met.
- Discuss the extent to which patient characteristics such as social class, race, and symptomology influence patient care.
- Identify and discuss ways in which the gender of the physician and the gender of the patient influence patient care.
- Discuss the issue of patient compliance with medical regimens from a sociological perspective.

Despite the increasing complexity of the health care system and the wide variety of healers and healing techniques that exist, the actual encounter between physician and patient remains a key element. Many people have an idealized picture of this relationship: A sick patient seeks comfort from a benevolent physician; the sincere and helpful patient places trust in the concerned and caring physician; both do whatever is necessary to restore health to the patient.

In fact, neither patients nor physicians are so uncomplicated or behave in such a uniform manner, and the relationship between the two can be an elusive phenomenon to diagram. As sociologists, our goal is to help clarify the relationships that actually develop between physicians and patients and identify important influences on the relationship.

MODELS OF THE PHYSICIAN–PATIENT RELATIONSHIP

The Parsonian Model

Nature of the Relationship. Within sociology, Talcott Parsons (1951) pioneered efforts at explaining the sociocultural foundation of health care. He viewed the physician–patient relationship as a subsystem of the larger social system. The key values in this subsystem reflected key values in society; they were shared by physicians and patients as they entered a relationship.

According to Parsons, the physician–patient relationship is inevitably (and fortunately) an asymmetrical one. Parsons believed that three circumstances dictated that physicians play the key, powerful role within the dyad and govern the relationship with patients.

1. **Professional prestige.** This is based on the physician's medical expertise, years of training, and the societal legitimation of the physician as the ultimate authority on health matters.
2. **Situational authority.** It is the physician who has established the medical practice and is offering her or his services to patients who have admitted their own inadequacies by soliciting the physician.
3. **Situational dependency.** It is the patient who has assumed the role of supplicant by seeking out service, scheduling an appointment, often waiting past the scheduled time, answering the physician's questions, and allowing an examination to occur.

Throughout each encounter, the "competency gap" between physician and patient is highlighted as the patient is dependent on the physician and the resources of the physician's office. However, Parsons expected that physicians would use their power wisely in promoting patients' best interests, and that patients would accept this arrangement as being the most efficient means to enact cure.

Freidson's Criticisms of the Parsonian Model. Perhaps the most important criticism of Parsons' model is that it overstates the "mutuality of interests" between physician and patient and does not provide for the considerable variation that now exists in physician–patient encounters. Conflict theorists dispute the notion that physicians and patients interact harmoniously and develop mutually satisfactory relationships through cooperation and consensus. Eliot Freidson (1970) has been a leading critic of the Parsonian model and an advocate for a conflict approach. He contends that conflict and dissensus are inevitable in any relationship in which the parties have such different backgrounds and power is so unequally distributed.

The Szasz–Hollender Model. An early (and now classic) effort to modify the Parsonian model was developed by two M.D.s, Thomas Szasz and Marc Hollender (1956). Arguing that Parsons gave too little attention to the important

influence of physiological symptoms, they developed their own typology of the physician–patient relationship, which includes three models.

The Activity–Passivity Model. This model closely parallels the asymmetrical relationship described by Parsons. The physician represents medical expertise, controls the communication flow between the two parties, and makes all important decisions. The patient is the supplicant, regarded as lacking in important information and necessarily relying on the knowledge and judgment of the physician. The relationship is akin to that of a parent and infant in which the parent takes actions without need of explanation.

The Guidance–Cooperation Model. Szasz and Hollender view this form of interaction as typical of most medical encounters. The patient is acknowledged to have feelings, may be alarmed by the medical problem, and has certain hopes and aspirations for the outcome of the medical encounter. Compared to the activity–passivity model, the patient has increased involvement in providing information and making decisions with regard to treatment. While the physician is still in charge and has responsibility for guiding the encounter, the cooperation of the patient is sought. The physician is less autocratic in the sense that some explanation is provided to the patient and the patient's assent to decisions is desired, but the physician retains the dominant position. Szasz and Hollender describe this relationship as being similar to a parent and adolescent.

The Mutual Participation Model. Based on a view that equalitarian relationships are to be preferred in medicine, this model elevates the patient to full participant. In this case, both physician and patient acknowledge that the patient must be a central player for the medical encounter to be successful. The patient knows more about her or his own situation—medical history, symptoms, and other relevant events—than does the physician. While the physician attempts to ask the proper questions to elicit key information, it is assumed that the patient also has an obligation to ensure that relevant information is disclosed.

In order for this type of relationship to work, Szasz and Hollender identify three essential traits that must be present. First, both participants must have approximately equal power (it is similar to a relationship between two adults); second, there must be some feeling of mutual interdependence (i.e., a need for each other); and finally, they must engage in interaction that will in some ways be satisfying to both parties.

Because this model “requires” more from the patient, they suggest it may be less appropriate for children or those who are mentally deficient, poorly educated, or very immature. On the other hand, those who are more intelligent or sophisticated, have broader experiences, and are more eager to take care of themselves may find this to be the only satisfying relationship.

KEY DIMENSIONS OF THE PHYSICIAN–PATIENT RELATIONSHIP

An appropriate model of the physician–patient relationship must acknowledge the considerable differences that exist among physicians and patients about what should occur within the relationship. The following three dimensions of the relationship are key:

1. The appropriate model of health (a belief in the biomedical or biopsychosocial model of health)
2. The primary ethical obligation of the physician (patient autonomy or beneficence)
3. The extent of commitment to and realization of genuine therapeutic communication

The actual relationship that develops between a given physician and a given patient is determined by the orientations held by both parties. This is not to deny that the physician is in the more powerful position. For the reasons enumerated by Parsons and elaborated upon by many others, physicians have the potential to command the decisive voice. But many physicians now reject this position, and many patients have been socialized not to let them assume it.

The Appropriate Model of Health

The Biomedical Model. As scientific discoveries produced meaningful explanations of diseases and effective medical treatments, the **biomedical model** of health became the dominant therapeutic orientation—a position it held for most of the last century. Biomedical medicine is essentially disease oriented or illness oriented rather than patient oriented. The key to effective medical care is believed to be correct diagnosis of some physiological aberration followed by proper application of the curative agent. Physicians seek to learn all they can about symptoms and abnormalities so that they can provide the appropriate “magic bullet.”

Consideration of social, psychological, and behavioral dimensions of illness has little place in this framework because it appears unnecessary. Engel (1977:129) cited one health authority speaking at a Rockefeller Foundation seminar who urged that “medicine concentrate on the ‘real’ diseases and not get lost in the psychosociological underbrush. The physician should not be saddled with problems that have arisen from the abdication of the theologian and the philosopher.” Another speaker had advocated “a disentangling of the organic elements of disease from the psychosocial elements of human malfunction.”

This biomedical focus has been reflected in medical education, which has surely helped to sustain it. Both coursework and clinical experience have emphasized the biological basis of disease and illness, while psychological and social factors have traditionally received little attention.

The Biopsychosocial Model. While there have always been individuals who lobbied for a broader-based approach to health care (George Engel, a professor of psychiatry and medicine at the University of Rochester Medical School, has been a key figure for much of his career), it was not until the 1970s that the campaign flourished. Engel argued that the benefits of the biomedical approach need not be sacrificed while incorporating psychosocial matters and that both are needed to provide optimal health care.

To provide a basis for understanding the determinants of disease and arriving at rational